

In-depth analysis of the public consultation of the European Parliament on rare diseases

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1. Introduction

Objectives

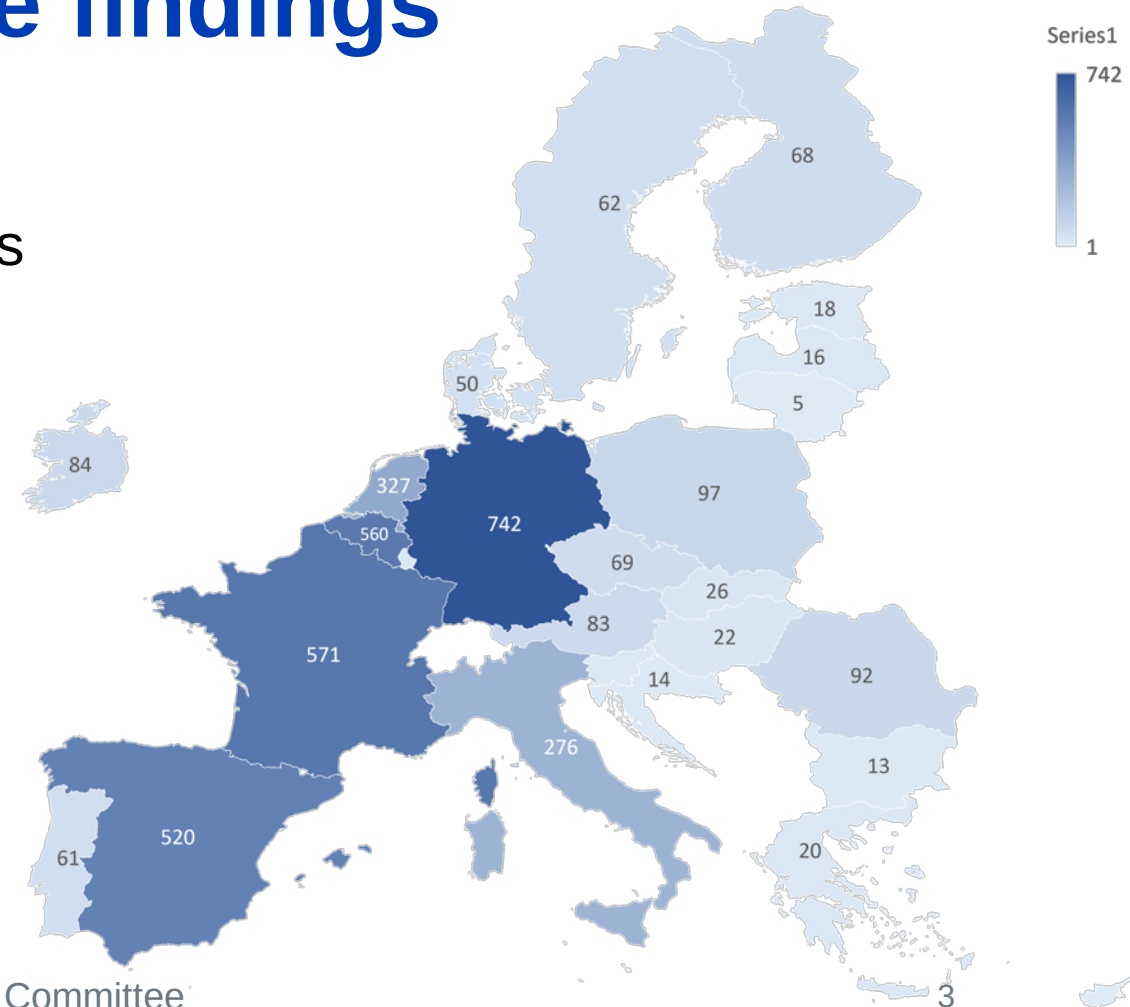
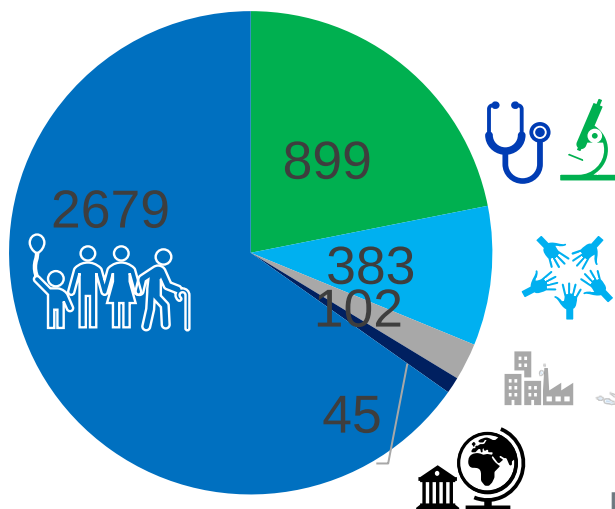
- To inform the forthcoming work of the Committee on Public Health
- By gathering insights into the challenges faced by people living with rare diseases as well as the views of professionals and stakeholders engaged in rare disease care and research.

Methodology

- Public consultation on rare diseases from 28/02/2025 - 31/03/2025
- Online questionnaire(s) developed by the European Parliament
- 5 respondent groups with adapted questions
- Analysis and report by Sciensano

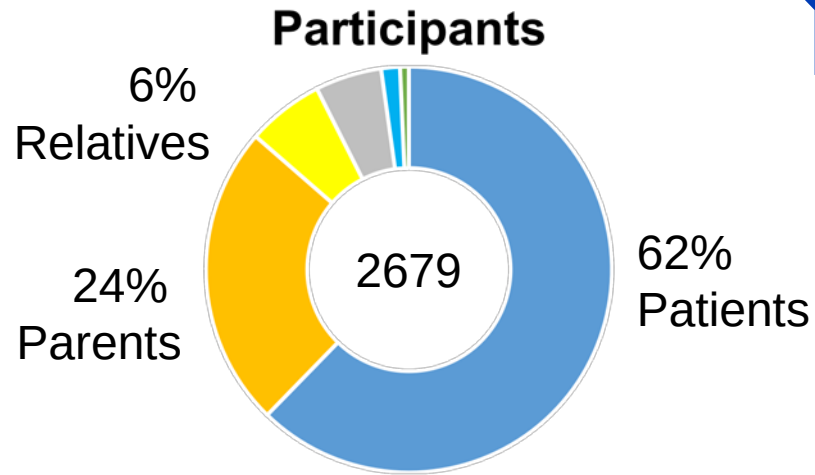
2.1. Quantitative findings

- 4,108 participants
- Participants across 27 EU Member States and outside of the EU
- Geographic imbalance in participation





Individuals

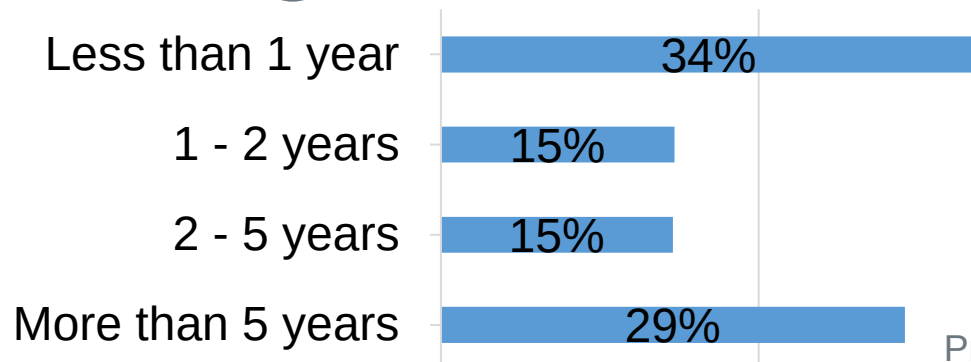


Impact on daily life

- 42% experienced difficulty working
- 39% struggled with household tasks
- 53% experienced discrimination



Time to diagnosis



Treatment exists for only 43%

- Only 5.2% received a curative therapy
- Access limitations: not accessible in the country (3% - 71 individuals); only partially (12%) or not reimbursed (4% - 93 individuals)



Individuals

Challenges:

1. Lack of specialists (48%)
2. Limited treatment options (37%)
3. Difficulty obtaining a diagnosis (29%)

Solutions:

1. Increase research funding (50%)
2. Better collaboration between HCP (39%)
3. More specialised centres (32%)

Regional differences in barriers to care:

- Eastern Europe: high concern across multiple areas, especially lack of specialists and high cost of care.
- Western Europe: barriers mainly systemic, including inadequate referrals and limited access to treatment options.



Patient support groups

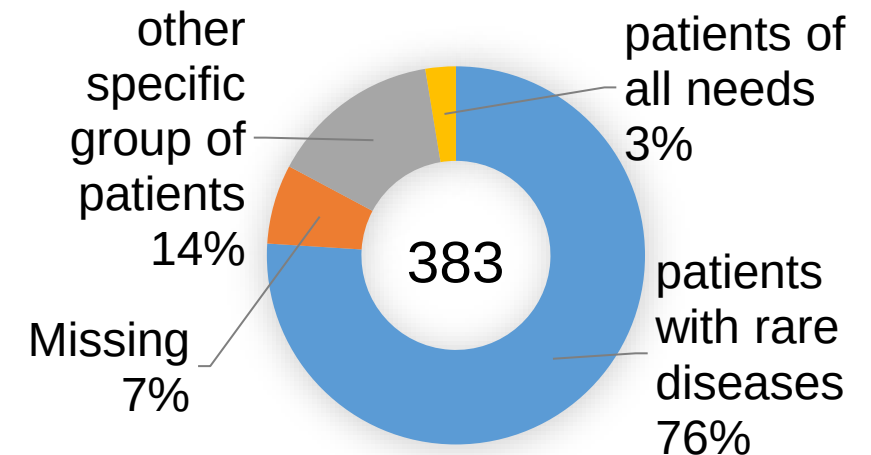
Scope

- 63.4% active in one EU Member State
- 23.2% active in multiple or all EU Member States

Challenges:

1. Difficulty in diagnosis (70%)
2. Access to support in daily life (65%)
3. Access to psychological support (65%)

Scope of the organisation





Healthcare professionals

Collaboration at national and international level was high (92% and 80%)

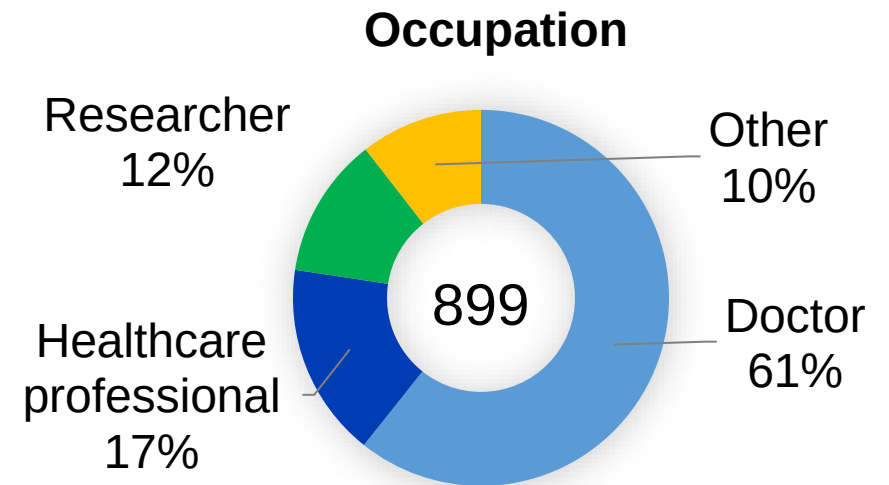
Challenges:

- Delayed diagnosis (77%)
- Insufficient research (62%)



Where the EU can improve:

1. Funding of networks (73%)
2. Increased research support (53%)
3. Legislative coordination (44%)



Key resources + need improvement

- specialised centres (68%)
- multidisciplinary teams (51%)
- rare disease registries (51%)



Stakeholder interest groups

Challenges: >50%

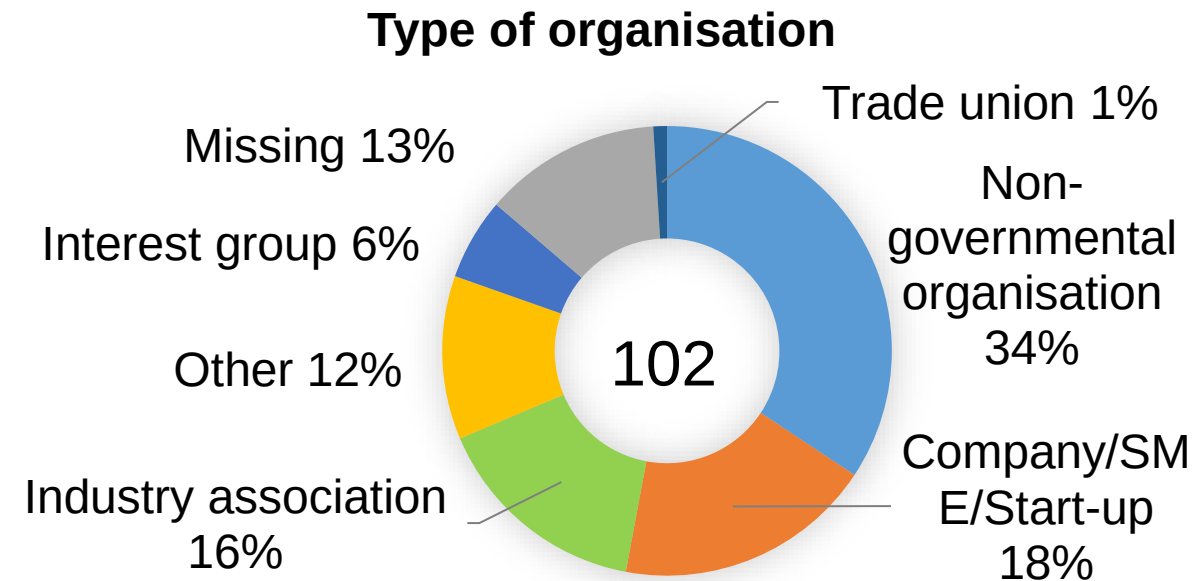
1. Difficulty in diagnosis (66%)
2. Limited access to treatment (58%)
3. Difficulty in accessing care (57%)



Where the EU can improve:

1. Cross-border healthcare (66%)
2. Research funding (64%)
3. Coordinating multinational research (59%)

81% agrees the EU has an important role in supporting rare disease research and treatment development





International organisation, government or other public body

Key challenges

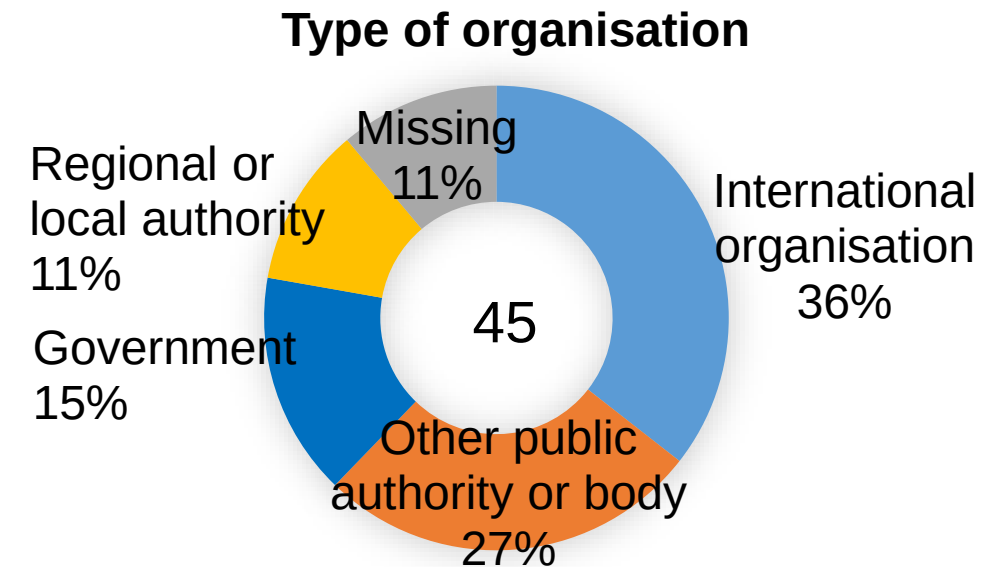
1. Data sharing (49%)
2. Low public awareness (44%)
3. Limited professional expertise (36%)



Proposed EU actions

1. Facilitating data sharing (67%)
2. Improving cooperation between research centres and patients (53%)
3. Harmonising legislation (47%)

53% of respondents view **joint purchasing** positively; 18% are neutral.



Awareness of EU initiatives

Repondent group	I am not aware of any EU action in rare diseases	European Reference Networks	ERDERA	European Experts group on rare diseases
1. Individuals	72.8% 	8.3% 	6.5%	8.1%
2. Patient support groups	8.3%	25.3%	17.0%	15.9%
3. Healthcare professionals	18.9% 	62.0%	31.3%	36.9%
4. Stakeholder interest groups	9.8%	64.7%	44.1%	43.1%
5. International organisations, government and other	6.7%	71.1%	46.7%	31.1%

2.2. Qualitative findings

Impact on daily life

Challenges:

- Impact on daily life, also for caregivers
- Discrimination in healthcare and society
- Lack of understanding and disbelief in healthcare and society
- Need for reasonable accommodations in the workplace and the education system
- Disability recognition
- Psychological support
- Social exclusion

 *“My daughter can hardly participate, she has a lot of therapy and that in combination with school is exhausting.” (individual)*

Awareness, empathy and knowledge

Lack of empathy and knowledge in the medical community was raised by patients and healthcare professionals

“*They don't know about my illness, if what they know is out of date, if they don't recognise many of the symptoms as being related to the illness, how can they help? If they don't bother to listen, how can they help?*” (individual)

Accessibility and equity

Ensure equitable access to screening, diagnosis, care and treatment for rare disease patients in every Member State.

Eliminate disparities based on geography or disease rarity.

Challenges:

- Care not available in home country
- Lack of specialists and specialised centres
- Long waiting times
- Long travel and medical deserts



"Significant disparities persist between Eastern and Western Europe in healthcare access, diagnostics and expertise. Western nations benefit from advanced systems and specialists, while Eastern countries face resource limitations, outdated infrastructure and workforce shortages. EU should properly address these discrepancies." (patient support group)

Care pathways and specialised centres

- Importance of referrals
- Importance of multidisciplinary, coordinated care



“Multidisciplinary teams should be employed all over EU for these patients (in Romania, usually 1 doctor handles a case, sometimes with no referral to a genetics specialist).” (healthcare professional)

Treatments

Challenges:

- Lack of availability of treatments
- Lack of reimbursement in EU MS
- No market authorisation in all MS
- Delayed EU approvals
- Drug withdrawals and shortages

Proposed solutions:

- Flexible regulatory pathways
- Incentive system for rare disease therapy
- Off-label and compassionate use regulation
- Framework for drug-repurposing
- Foresee cross-border clinical trials



*"As a rare disease patient, I took part in a clinical trial. I was put on new medication for 3 months, it was life changing, the improvement was immediate & they've kept me out of hospital. Once the trial finished so did the supply of medication. I now **travel to another EU country to buy the medication** (at great expense). They should be available in my country." (individual)*



European reference networks

- Sustain, adequately support, and finance ERNs
- Need for enhanced collaboration with non-ERN members
- ERNs are also valuable for research, guideline development, education and communication
- Address gaps in specialised centres and uneven geographical coverage

 *"Please **revise the ERN structure**: generate 1 entry point per country, but allow local specialized centres to be affiliated for their expertise. We have the situation in Belgium where for a certain ERN 2 national centres are ERN-centres and other large centres are not ERN centres, but have more expertise for certain specific diseases within the ERN." (healthcare professional)*

3. Key takeaways & conclusions

Major challenges

- Delayed and unequal diagnosis
- Limited access to treatments
- Availability of specialists
- Insufficient research and funding
- Social and psychological burden

Recommendations

- Adopt a EU Rare Disease Plan + national follow-up
- Harmonise access to care across Member States
- Strengthen ERNs and cross-country collaboration
- Harmonise legislation and incentivise treatments
- Support research, clinical trials, registries and data sharing
- Promote patient involvement and social inclusion

Promising EU actions exist, but sustained effort is needed to turn policies into tangible improvements for all rare disease patients across Europe.

Thank you for your attention

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